



RISK ASSESSMENT

EBOLA - BUNDIBUGYO OUTBREAK DRC & UGANDA

Risk Assessment published 8/6/2026 <https://www.sciensano.be/en/projects/coordination-risk-assessment-group>.

RAG contact: Jorgen Stassijns (+ 32 478 84 02 91 or jorgen.stassijns@sciensano.be) and rag@sciensano.be

SIGNAL

On 15 May 2026, Africa CDC reported an outbreak of Ebola virus disease (EVD) in Ituri Province, DRC.

DESCRIPTION

Event

On 15 May 2026, Africa CDC reported an outbreak of Ebola disease in **Ituri Province, DRC**. Laboratory testing by the Institut National de Recherche Biomédicale (INRB) identified the **Bundibugyo species**, with 8 of 13 samples testing positive. According to WHO AFRO and Africa CDC, as of 16 May 2026, 246 suspected cases and at least 80 suspected deaths had been reported across three health zones (Bunia, Mongwalu, Rwampara), including four deaths among confirmed cases.

On 17 May 2026, WHO declared the Bundibugyo Ebola outbreak a **Public Health Emergency of International Concern (PHEIC)**. The following day, Africa CDC declared a **Public Health Emergency of Continental Security**.

Outside DRC, **Uganda** reported two cases of confirmed Ebola Bundibugyo on 15 and 16 May 2026. Both were imported cases detected in Kampala.

Epidemiological situation (as of 31 May) (Annex 1): The number of suspected cases has **decreased** following investigations and testing, which confirmed some cases and ruled out others. **Suspected deaths** have been **temporarily removed** from the count while investigations are still underway.

- In **DRC**, **321** confirmed cases, including **48** deaths, have been reported across three affected provinces: **Ituri** (299 cases), **North Kivu** (19), and **South Kivu** (3). An additional 116 **suspected cases** are under investigation, while six confirmed cases have **recovered**. A total of **23 health zones** are affected across the three provinces.
- In **Uganda**, **15** confirmed cases, including **one** death, have been reported to date (in Kampala and one in Wakiso). As of 1 June, **668 contacts** had been identified and are under follow-up.

Several **symptomatic travellers** from affected areas have been tested in European and non-European countries: all results have been **negative** so far. A **positive case**, an American citizen working in healthcare in the affected areas was transferred with six high-risk contacts and is treated in Germany.

Type of risk for DRC & Uganda

Expected and unusual

Since its discovery in 1976, this is the 17th Ebola Disease outbreak in the DRC. The most recent prior outbreak occurred in 2025 in Kasai Province due to Ebola virus (species *Orthoebolavirus zairense*). In Ituri Province specifically, Ebola disease due to Ebola virus (*Orthoebolavirus zairense*) was last documented during the 2018–2020 outbreak; this outbreak was declared on 1 August 2018 following reports of laboratory-confirmed cases in North Kivu Province. Investigations identified cases in Ituri and North Kivu and the outbreak also spread to South Kivu. At the time, WHO declared the outbreak a Public Health Emergency of International Concern.

Although Ebola outbreaks occur regularly in DRC, **outbreaks due to Bundibugyo virus are unusual events**. Bundibugyo virus (BDBV) was first reported in 2007 in Bundibugyo district in Uganda and a second outbreak was reported in DRC in 2012. **This is the third outbreak of Bundibugyo ever recorded and the second in DRC.**

Several factors are affecting the (spread of the) outbreak locally:

- Delays in verification of initial signal by authorities and retrospective identification of cases and deaths;
- Epidemiological links and the full chain of transmission not yet clearly established;
- Challenges concerning testing capacity: Genexpert platforms that are normally widely used, don't detect BDBV which results in difficult upscaling of the diagnostic capacity;
- Affected area characterized by intense population mobility linked to mining activities, trade, and movement between rural and urban centres, leading to a rapidly expanding outbreak;
- Weak infection and prevention control (IPC) practices in many health facilities;
- Affected provinces of Ituri and North Kivu being highly insecure, with intensified fighting in recent months, restricting the movement of surveillance teams, limiting the deployment of Rapid Response Teams, and hampering the secure transport of laboratory samples, as well as challenges in contact tracing;
- Significant distrust of health and external authorities among the local population;
- Limited healthcare infrastructure and inadequate isolation capacity;
- Unsafe burial practices;
- No licensed vaccine or specific therapeutics are currently available for prevention and treatment;
- Epicentre of outbreak occurring in a region neighbouring two other countries.

Severity of the risk

High

Severity is considered **high** as:

- **Case fatality rates vary between 25-90%** depending on the type of orthoebolavirus and whether supportive care is available. For BDBV, case fatality rates in the past two outbreaks, reported in Uganda and in DRC in 2007 and 2012, have ranged from approximately **30% to 50%** (estimations based on a limited number of cases). Patients have a much better chance of surviving if they receive early supportive care.

- There are currently **no approved treatments against BDBV nor licensed vaccines**. The control of the outbreak should therefore focus on the early detection and isolation of cases. It is unclear if and how much cross-protection the vaccine against EVD due to Zaire ebolavirus would provide against EVD due to Bundibugyo ebolavirus, since the viruses belong to different species and existing cross-protection data remain limited and inconclusive.

Exposed population

Although information about the current outbreak is still limited, [ECDC](#) considers the **likelihood of being exposed to Ebola virus for people from Europe travelling to or living in affected areas as being low**, provided they follow recommended precautions. Ebola disease spreads through direct contact with the blood or other bodily fluids of people or animals with the infection (living or dead). This type of exposure is unlikely for most travellers or expatriates living in the affected areas.

[ECDC](#) considers that **healthcare workers and humanitarian staff** working directly with patients or local communities in affected areas are more likely to be exposed to the virus. However, if appropriate infection prevention and infection control measures are followed, the risk of exposure for these groups is also considered **low**.

Risk of international dissemination

Very low

Based on the currently available epidemiological information:

The risk of spreading at international level (Europe):

- The most likely way the virus could spread and reach Europe would be through people with an infection travelling from affected areas to Europe. However this is **unlikely**, given the current location of the outbreak, and the response activities in place. Moreover, based on experience from the Ebola outbreak in West Africa between 2013 and 2016 – the largest outbreak to date – imported cases into Europe are expected to be very rare. During that outbreak, despite tens of thousands of cases and large international deployments to affected countries, only a small number of cases were reported in Europe, most of which were those who had been medically evacuated for treatment¹. Finally, no travel-associated EVD cases have been reported among travellers returning to Europe during recent outbreaks in Africa (2018-2025).
- The likelihood of **secondary transmission** of BDBV within Europe and the occurrence of sustained chains of transmission within the European countries is considered **very low**, as cases are likely to be promptly identified and isolated and recommended control measures would be implemented.

The risk of spreading at national level (DRC and Uganda):

- [WHO](#) assesses the risk of further spread of this event at the national level as very high due to the rapid geographic expansion and increasing number of affected health zones within a short period.
- While retrospective investigations initially identified cases across three health zones in Ituri Province (Mongbwalu, Rwampara, and Bunia), as of 31 May 2026 suspected and confirmed cases have been reported from 23 health zones, including 15/36 in Ituri Province, 7/35 in North Kivu Province and 1/34 in South Kivu.
- In addition, Uganda has reported 15 laboratory-confirmed cases in Kampala, further confirming cross-border transmission and increasing the risk of regional

¹ https://www.ecdc.europa.eu/sites/default/files/documents/Ebola%20and%20Marburg%20fevers%20AER_0.pdf

spread. Population movement between mining areas, rural communities, and urban centres is frequent, and Bunia serves as a major transport, referral, and commercial hub connected to other provinces and neighbouring countries, supporting the spreading of the diseases.

Although the number of cases is increasing and the outbreak rapidly expanding geographically, the government of the Democratic Republic of the Congo (DRC), alongside the World Health Organization (WHO), Africa CDC, and international humanitarian partners, are mobilising an intensive containment strategy. Response measures are in place and include deployment of rapid response teams (identification of cases and contact tracing), delivery of medical supplies, strengthened surveillance, laboratory confirmation, infection prevention and control assessments, the set-up of safe treatment centres, community engagement and exit screening.

PREPAREDNESS & CONTROL MEASURES ALREADY IN PLACE

General preparedness

Protocols and guidelines:

- [National Procedure for Case Management](#) VHF published on the FOD/SPF website.
- Draft Viral Outbreak Plan (VOP) developed by the FOD/SPF.
- Fiche [Maladie Infectieuse-Ebola](#) available from AVIQ.
- [Virale hemorrhagische koorts](#) available from Departement Zorg.
- Fiche [Ebola virus disease](#) available from Sciensano.

Diagnosis and surveillance:

- The diagnostic tests are carried-out at the Institute of Tropical Medicine (ITM) in its role as national reference laboratory for the diagnosis and treatment of infections and tropical diseases ([MB 31/08/1998-BS 10/12/1998](#)); the diagnostic capacity is estimated at 6 tests/run.
- Epidemiological surveillance carried-out by Sciensano.

A national, multisectoral **Ebola-simulation exercise** was organized by the FPS Public Health with support from ECDC/EU Health Task Force on 17/4/2026. An after action report was shared with all stakeholders. Simulation exercises were also carried out in 2026 at both reference hospitals (UZ Antwerp and CHU Saint-Pierre, Brussels).

Specific Control Measures

Several specific measures have been put in place in Belgium since the start of the outbreak, such as:

- The National Crisis Centre (NCCN) organised coordination meetings on 22 and 29 of May with all sectors involved.
- The FOD/SPF Public Health published [recommendations, procedures and practical information](#) for health care providers. A letter was sent to hospitals and general practitioners to inform them of the situation and available resources on the website.
- The FOD/SPF Public Health updated the [Ebola webpage](#) for the general public.
- The FOD/SPF Public Health developed a communication strategy for the general public in case of a positive case in Belgium. Communication materials are also available for the positive case and their contacts.

- The federated entities are conducting awareness campaigns for the Congolese community in Belgium.
- Procedures at points of entry have been strengthened: information for travellers (information posters/banners) is available at the airport and Saniport staff is present when flights from the affected countries arrive (DRC, Uganda), to inform passengers and take care of potential suspected cases (procedure available). Awareness raising among airline staff is ongoing.; Systematic screening of returning travellers (visual or via temperature checks) was considered but not implemented because of lack of evidence concerning the efficiency of such measures.
- Epidemiological information on the outbreak will be published through the News Flash of June, targeting general practitioners, infectiologists and diagnostic laboratories.

PUBLIC HEALTH IMPACT IN BELGIUM

Very low

- Due to the very recent declaration of the outbreak and the uncertainties related to the epidemiological situation, it is probable that the outbreak is larger than what is currently being reported – not only in regards to the number of affected cases, but also in its geographical extent. There are also concerns because there are currently no licensed vaccines or specific treatment for Bundibugyo virus.
- However, based on past experience, the **importation of an infected traveller in Belgium is considered as very unlikely** and, should that happen, the likelihood of further transmission and **risk for the Belgian population is considered very low**.
- The repatriation of an infected humanitarian worker in Belgium is possible given the considerable involvement of organisations such as Médecins Sans Frontières (headquarters in Brussels) or ITM.
- The likelihood of infection for people from **Belgium living in or travelling to affected areas is assessed as low**, provided they adhere to the recommended precautionary measures.

RECOMMENDATIONS

- Alignment with the ECDC assessment regarding the affected areas/regions in DRC and Uganda. Based on the current available evidence the RAG does not recommend to consider the whole of DRC as country at risk of infection.
- Provide targeted information to first line healthcare providers – general practitioners and hospital emergency wards- through (i) the organisation of a webinar presenting the National Procedure for Case Management VHF; (ii) the availability of operational and concise guidelines based on the National Procedure for Case Management VHF.
- Coordinated actions related to the deployment/return of (humanitarian) workers to the affected areas
- Follow-up the capacity for upscaling, in particular the stock of PPE.
- Check the availability of an experimental treatment in case of a confirmed case.
- Ensure a common access to up-to-date epidemiological information for all stakeholders
- Surveillance of BDBV in wastewater is currently not recommended. In the current situation, there is no evidence that such surveillance (from wastewater

at the airport or from an aircraft arriving from the affected countries) would offer an added value for early detection or for implementation of measures.

ACTIONS

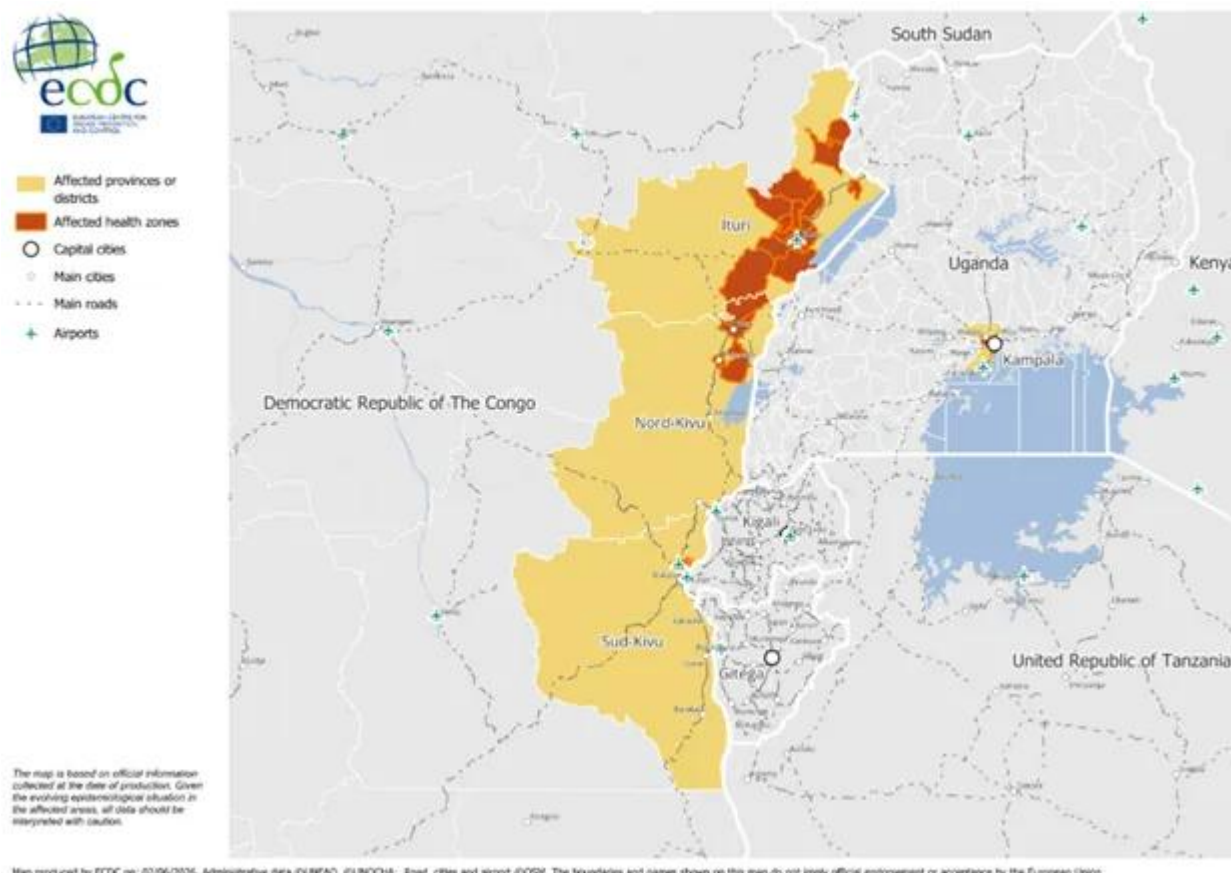
- Epidemiological follow-up: Sciensano.
- Addition of links for updates of the epidemiological situation in FOD Dashboard: Sciensano to provide shortlist to FOD/SPF
- Organisation of a webinar for general practitioners and hospital emergency wards to present the National Procedure for Case Management VHF : RMG/federated entities
- Compilation of operational guidelines linked to the National Procedure for Case Management VHF available from reference hospitals and sharing with healthcare providers: lead to be discussed at RMG
- Check availability of stocks of PPE: RMG
- Contact WHO to check availability of experimental treatment: responsible to be decided.

REFERENCES

- **ECDC Weekly update** : <https://www.ecdc.europa.eu/en/ebola-outbreak-democratic-republic-congo-and-uganda>
- **ECDC daily RT**
- **ECDC threat assessment brief**: <https://www.ecdc.europa.eu/en/publications-data/threat-assessment-brief-ebola-disease-outbreak-caused-bundibugyo-virus-democratic>
- **INSP - Sitrep Ebola de la RDC** : <https://insp.cd/ebola-17eme-epidemie/>
- **MoH Uganda - Ebola press statements** : <https://health.go.ug/downloads/press-statements/>
- **WHO African Region weekly update** : <https://www.afro.who.int/health-topics/ebola-disease/outbreak-drc-26>
- **FPS Health: VHF outbreak protocol**:
 - FR: <https://www.health.belgium.be/fr/organisation-politique/reglementation-documents-politique/procedure-gestion-risques-lies-aux-fievres-hemorragiques-virales-destination-professionnels-sante>
 - NL: <https://www.health.belgium.be/nl/organisatie-beleid/regelgeving-beleidsdocumenten/procedure-risicobeheer-virale-hemorragische-koorts-gezondheidswerkers-andere-relevante-stakeholders>
- **FPS Health: information for healthcare professional, VHF**:
 - FR: <https://www.health.belgium.be/fr/professionnels/professionnels-sante/sante-humaine/gestion-risques-crises-sanitaires/lutte-contre-maladies-infectieuses/maladies-infectieuses/fievres-hemorragiques-virales-dont-ebola-lassa-crimee-congo>
 - NL: <https://www.health.belgium.be/nl/professionals/gezondheidsprofessionals/menselijke-gezondheid/beheer-risicos-gezondheids crisissen/bestrijding-infectieziekten/infectieziekten/virale-hemorragische-koorts-ebola-lassa-crimean-congo>

ANNEXES

ANNEX 1: MAP AFFECTED REGIONS



ANNEX 2: METHODOLOGY

DESCRIPTION METHODOLOGY	
EVIDENCE	
AVAILABILITY OF EVIDENCE	This risk assessment was made based on available evidence from ECDC, WHO, Africa CDC and INSP. Because of the lack of diagnostic capacity, the lack of access to health care, the instability of the region and the rapidly evolving situation, this assessment might change once new epidemiological evidence becomes available.
EXPERTS	
SELECTION PROCESS	Experts were (pre) selected based on expertise, motivation letter, current position, as well as representatives of permanent members. The full list of RAG experts is available at https://www.sciensano.be/en/rag-experts .
EXPERT LIST (<i>alphabetic order</i>)	Leïla Belkhir (Saint Luc/UC Louvain), Caroline Boulouffe (AVIQ), Isabel Brosius (ITM), Natalia Bustos Sierra (Sciensano), Steven Callens (UZ Gent), Anthony De Backer (AVIQ), Géraldine De Muylder (Sciensano), Pierre-Louis Deudon (FAMGB/CMG), Naima Hammami (DZ), Bart Hoorelbeke (FOD VVVL - SPF SPSCAE), Sylvie Jonckheere (MSF), Laurens Liesenborghs (ITM), Charlotte Martin (CHU Saint-Pierre), Christelle Meuris (CHR Verviers), Sophie Quoilin (CHU Saint-Pierre), Javiera Rebolledo Romero (Sciensano), Florence Rolin (Vivalis), Carole Schirvel (St Luc/UC Louvain), Anna Schmelz (DGOV), Jorgen Stassijns (Sciensano/RAG), Gerlant Van Berlaer (FOD VVVL - SPF SPSCAE), Marjan Van Esbroeck (ITM), Erika Vlieghe (U Antwerp), Liesbeth Van Gestel (DZ).
DECLARATION CONFLICT OF INTEREST	For those experts who have submitted a DOI, no conflict of interest was found. For those who have not yet submitted a DOI, a declaration of conflict of interest will be requested.
CONSULTATION PROCESS	
MEETING	Draft Risk Assessment sent to experts on 3/6/2026 Online meeting with experts on 3/6/2026 Final draft approved via mail on 4/6/2026 Final RA published 8/6/2026
SURVEY	No survey was carried-out among experts
DECISION PROCESS	
CONSENSUS	Consensus on the final version on 4/6/2026.
OBJECTIONS	No major objections were expressed by the experts.